

**PATIENT CHART -
BUTLER, ALVA
5316 FISHERSOUND LN
APOLLO BEACH FL 33572
O: (260) 438-4646(preferred)**

DOB: 10/4/1940 AGE: 85 yrs. Acct#: 6227



Alva Butler
5316 FISHERSOUND LN
APOLLO BEACH, FL 33572
(260) 804-1891

Yvette Guzman
4912 West Trapnell Rd Plant City, FL 33566-0128
Phone: (813) 417-4767 Fax: (888) 814-0945
VisitDate:04/13/2026

DOB:10/04/1940

85 year old, male

Race:No race saved for patient

Ethnicity:No ethnicity for patient saved

PreferredLanguage:English

CurrentAllergies:

NKDA

CurrentMedications:

Novolog FlexPen U-100 Insulin,
100 unit/mL (3 mL)

acetaminophen, 325 mg

Senexon-S, 8.6-50 mg

oxycodone, 5 mg

alprazolam, 0.25 mg

thiamine mononitrate (vit B1),
100 mg

tamsulosin, 0.4 mg

rosuvastatin, 40 mg

polyethylene glycol 3350, 17
gram/dose

Nutren 2.0, 0.08 gram-2 kcal/mL

levothyroxine, 50 mcg

isosorbide mononitrate, 20 mg

Flonase Allergy Relief, 50
mcg/actuation

clopidogrel, 75 mg

Eliquis, 5 mg

diltiazem HCl, 90 mg

dapagliflozin propanediol, 5 mg

famotidine, 20 mg

carvedilol, 3.125 mg

Date of contact: 04/07/2026**Date of ED visit:** 03/18/2026**ED Discharge Date:**

04/01/2026**Care Team:** Yvette Guzman APRN DNP

Njedika Nnadozie APRN

Jennifer Morris CCMA**How are you feeling after discharge:** better, nutrition

improving with PEG tube**Were you given any prescriptions?** No**Do you have**

any pending labs or tests? No**Do you have any questions or concerns?:**

No**Needs follow up appointment Within 14 days of discharge:** ☒ **Appointment**

Scheduled: 04/13/2026**Additional Information Needed and Requested:** none**Do**

you have any questions or concerns?: No**Needs follow up appointment Within**

14 days of discharge: ☒ **Additional Information Needed and Requested:**

none**SocialHistory:**

none

History of Use

TOBACCO USE SCREENING

Smoking Status: Smoking Status Not Recorded

Smoked Per Day: Less Than 1

Smoking Effects

EDUCATION GIVEN ABOUT:

STOP SMOKING MONITORING INVITATION GIVEN

REFERRAL TO STOP-SMOKING CLINIC: na

ALCOHOL USE: Patient denies alcohol use.

DRUG USE: Patient denies drug use.

CAFFEINE: Patient denies caffeine use.

EMPLOYMENT: Retired

SEXUAL ACTIVITY: Sexually active in the past with 2

CONTRACEPTION: na

LabResults:

Test
XRay

Result

Abn Flg

Performed
04/30/2026

Subjective

ChiefComplaint:Establish care

HistoryofPresentIllness:

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Visit Date: 04/13/2026

Alva is an 85yrs/M with T2 Diabetes Mellitus and Hypothyroidism residing at home with daughter. He ambulates with the aid of both walker and wheelchair. Patient is seen today for care transmission. Daughter was present during the visit and reported that patient was discharged home from rehabilitation two weeks ago. Patient was admitted into the rehab for physical therapy and nutrition management.

Patient is following with woundology for his sacral pressure wound. He is also following with physical/occupational therapy. He has home healthcare visits twice weekly.

He has a PEG tube and is feed with Neutron 2.0 at 250 ml every eight hours. He reports good tolerance to his feeds, sleeping well with regular bowel movements. He denies any gastrointestinal disturbances, shortness of breath or headaches.

On examination, patient was seen resting on his bed, calm, alert and oriented. He was not in any obvious distress. Left hand bruising was observed and when asked to make a fist, patient was able to make a fist with the right hand and not with the left hand. Drug review and reconciliation was completed with the daughter.

Objective:

Pulse: 67

Respiration: 24

O2 Sat Room Air: 97%

BP 1: 91 / 44

Assessment

E11.65 Type 2 diabetes mellitus with hyperglycemia

E03.9 HYPOTHYROIDISM, UNSPECIFIED

M62.562 MUSCLE WASTING AND ATROPHY, NEC, LEFT LOWER LEG

M62.561 MUSCLE WASTING AND ATROPHY, NEC, RIGHT LOWER LEG

Review of Systems

Positive and pertinent negative responses below. All other areas negative.

Alva Butler DOB: 10/04/1940 85 year old, male
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Visit Date: 04/13/2026

Gastrointestinal - PEG Tube.
Musculoskeletal - muscular weakness, muscular atrophy, range of motion limitations.
Neurological - muscular weakness, coordination difficulty.
Endocrine - Hypothyroidism, Diabetes Mellitus.

Problem List:

No known problems

Plan

BMI Follow-up Plan: reviewed, discussed
Nutrition Counseling: discussed, education given
Physical Activity Counseling: discussed, education given
T2DM - Continue Novolog sliding scale subQ daily as directed.
Ensure regular blood glucose monitoring, annual eye examination and regular foot checks.
Encourage carb consistent diet and medication compliance and adherence.

Hypothyroidism - Continue Tab levothyroxine 50 mcg, oral once daily.
Monitor thyroid labs (TSH, free T4), vital signs, and symptoms.
Administer levothyroxine consistently; educate on interactions and adherence.
Supportive care: manage fatigue, constipation, dry skin, and cold intolerance.
Teach recognition of under- and over-treatment signs.
Stress lifelong therapy, regular follow-up, and awareness of myxedema coma risk.

To follow up in one month.

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Visit Date: 04/13/2026

Electronically Signed: Yvette Guzman, APRN DNP on/at 05/13/2026 09:22:22

Signed By: Yvette Guzman, APRN DNP 05/13/2026 09:22 AM



Alva Butler
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 (260) 804-1891

Melissa Cole
 4912 West Trapnell Rd Plant City, FL 33566-0128
 Phone: (813) 417-4767 Fax: (888) 814-0945
 Visit Date: 06/19/2026

DOB: 10/04/1940
 85 year old, male
Race: No race saved for patient
Ethnicity: No ethnicity for patient saved
Preferred Language: English

Current Allergies:
 NKDA

Current Medications:
 Novolog FlexPen U-100 Insulin,
 100 unit/mL (3 mL)
 acetaminophen, 325 mg
 oxycodone, 5 mg
 alprazolam, 0.25 mg
 thiamine mononitrate (vit B1),
 100 mg
 tamsulosin, 0.4 mg
 rosuvastatin, 40 mg
 polyethylene glycol 3350, 17
 gram/dose
 Nutren 2.0, 0.08 gram-2 kcal/mL
 levothyroxine, 50 mcg
 isosorbide mononitrate, 20 mg
 Flonase Allergy Relief, 50
 mcg/actuation
 clopidogrel, 75 mg
 Eliquis, 5 mg
 diltiazem HCl, 90 mg
 dapagliflozin propanediol, 5 mg
 famotidine, 20 mg
 carvedilol, 3.125 mg
 megestrol, 400 mg/10 mL (40
 mg/mL)
 lactulose, 10 gram/15 mL
 Senexon-S, 8.6-50 mg
 Bactrim DS, 800-160 mg
 mupirocin, 2 %
 sodium chloride, 1,000 mg
 levofloxacin, 250 mg
 Thick-It

Lab Results:

Test results	Result	Abn Flg	Performed
	---		06/22/2026

Subjective

Chief Complaint: lab review, choking with liquids

History of Present Illness:

Alva is an 85yrs/M with multiple chronic disordered who residing at home with daughter.
 History was obtained primarily from the patient's daughter, Deborah, during today's telehealth visit. received call from HH that he is having some bruising that waxes and wanes she has been concerned about him being on clopidigrel and eliquis. He does have paroxysmal atrial fibrillation and has multiple stents in. Since his condition has worse, he has not been followed by a cardiologist. His symptoms remain the same as discussed two days ago when our labs were older. The only symptom is that the daughter is worried that he may be aspirating when he drinks fluids. She said that he was in the rehab. They did a swallow study because he was doing the same and they had him however, when they discharged, they said he did not need to continue that after discharge. Also lab review today that was ordered during telehealth visit two days ago. abnormals As follows:

Na 133L
 K 5.7H
 Glucose 232 (not fasting)
 Hgb 15.7/Hct 48.7
 creatinine WNL, GFR 56, BUN 29H
 Urine 4+ glucose, blood 3+, leuk positive, protein 2+, nitrites neg
 PCR pseudomonas aeruginosa

Requesting cardiology referral. awaiting urine C&S. she also states she did not start the bactrim that was ordered previously. no acute distress today.

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Visit Date: 06/19/2026

Medical History: P afib, CAD, DM2, hyperlipidemia, hypertension, hypothyroidism, prostate cancer, constipation, squamous cell carcinoma, the mouth/tongue with mets, congestive heart failure, dilated cardiomyopathy

Objective:

Assessment

E11.65 TYPE 2 DIABETES MELLITUS WITH HYPERGLYCEMIA
 N39.0 URINARY TRACT INFECTION, SITE NOT SPECIFIED
 R13.10 DYSPHAGIA, UNSPECIFIED
 R41.82 ALTERED MENTAL STATUS, UNSPECIFIED
 E87.1 HYPO-OSMOLALITY AND HYPONATREMIA
 R05.9 COUGH, UNSPECIFIED
 N18.31 CHRONIC KIDNEY DISEASE, STAGE 3A
 I25.10 ATHSCL HEART DISEASE OF NATIVE CORONARY ARTERY W/O ANG PCTRS
 I48.0 PAROXYSMAL ATRIAL FIBRILLATION

Review of Systems

Positive and pertinent negative responses below. All other areas negative.
 Constitutional - gen weakness, at baseline.
 Cardiovascular - denies sympoms.
 Respiratory - slight congested cough.
 Gastrointestinal - constipation managed.
 Genitourinary - denies but abnormal urine result.
 Skin - gtube site slight improvement.
 Psychiatric - still having some confusion.

Problem List:

Code	Description	Onset	Status	Status Date
E03.9	HYPOTHYROIDISM, UNSPECIFIED	05/13/2026	Current	05/13/2026
E11.65	TYPE 2 DIABETES MELLITUS WITH HYPERGLYCEMIA	05/13/2026	Current	05/13/2026
E43	UNSPECIFIED SEVERE PROTEIN-CALORIE MALNUTRITION	06/22/2026	Current	06/22/2026
K59.00	CONSTIPATION, UNSPECIFIED	06/16/2026	Current	06/16/2026

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Visit Date: 06/19/2026

M62.561	MUSCLE WASTING AND ATROPHY, NEC, RIGHT LOWER LEG	05/13/2026	Current	05/13/2026
M62.562	MUSCLE WASTING AND ATROPHY, NEC, LEFT LOWER LEG	05/13/2026	Current	05/13/2026
N39.0	URINARY TRACT INFECTION, SITE NOT SPECIFIED			06/16/2026
		Current		06/16/2026
R60.0	LOCALIZED EDEMA	06/22/2026	Current	06/22/2026

Plan

BMI Follow-up Plan: reviewed, discussed

Nutrition Counseling: discussed, education given

Physical Activity Counseling: discussed, education given

Urinary Tract Infection: changing antibiotic to levofloxacin per PCR. she did not pick up the bactrim ordered based on previous PCR E. coli. meds as ordered if prescribed. increase intake of fluids. practice appropriate perineal hygiene. report increased pain or discomfort, pain in back, or fever.

dysphagia/cough: ordering chest xray. monitor for worsening symptoms and report. 911 for shortness of breath. ordered thick diet from pharmacy will try nectar thick

hyponatremia: ordering sodium tablets for a week while they are increasing fluids via tube as to not cause more dilution and lower the level

DM2- continue current treatment. avoid concentrated sweets and consuming large amounts of carbohydrates, try to get 30 min of activity daily. if checking blood sugar, report reading above 300 or less than 60. report increased urination, hunger or thirst. will monitor labs and adjust treatment accordingly. likely elevated due to infection.

CKD- difficult for him to leave the house but will consider nephrology referral if renal function continues to decline. avoid NSAIDs and increase fluid intake

Dehydration: double water intake via feeding tube for the next week.

CAD/ afib: lowering eliquis to 2.5mg bid. referral to mobile cardiology sent. please continue to monitor for bleeding or bruising.

CG agrees with plan and verb understanding

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Visit Date: 06/19/2026

Electronically Signed: Melissa Cole, APRN DNP on/at 06/26/2026 11:38:49

Signed By: Melissa Cole, APRN DNP 06/26/2026 11:38 AM



Alva Butler
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Melissa Cole
 4912 West Trapnell Rd Plant City, FL 33566-0128
 Phone: (813) 417-4767 Fax: (888) 814-0945
 Visit Date: 06/22/2026

DOB: 10/04/1940
 85 year old, male
Race: No race saved for patient
Ethnicity: No ethnicity for patient saved
Preferred Language: English

Current Allergies:
 NKDA

Current Medications:
 Novolog FlexPen U-100 Insulin,
 100 unit/mL (3 mL)
 acetaminophen, 325 mg
 oxycodone, 5 mg
 alprazolam, 0.25 mg
 thiamine mononitrate (vit B1),
 100 mg
 tamsulosin, 0.4 mg
 rosuvastatin, 40 mg
 polyethylene glycol 3350, 17
 gram/dose
 Nutren 2.0, 0.08 gram-2 kcal/mL
 levothyroxine, 50 mcg
 isosorbide mononitrate, 20 mg
 Flonase Allergy Relief, 50
 mcg/actuation
 clopidogrel, 75 mg
 Eliquis, 5 mg
 diltiazem HCl, 90 mg
 dapagliflozin propanediol, 5 mg
 famotidine, 20 mg
 carvedilol, 3.125 mg
 megestrol, 400 mg/10 mL (40
 mg/mL)
 lactulose, 10 gram/15 mL
 Senexon-S, 8.6-50 mg
 Bactrim DS, 800-160 mg
 mupirocin, 2 %
 sodium chloride, 1,000 mg
 levofloxacin, 250 mg
 Thick-It

Lab Results:

Test results	Result	Abn Flg	Performed
	---		06/22/2026

Subjective

Chief Complaint: review chest xray and urine

History of Present Illness:

Alva is an 85yrs/M with multiple chronic disordered who residing at home with daughter.
 History was obtained primarily from the patient's daughter, Deborah, during today's telehealth visit.. today we are reviewing his chest x-ray results.
 There's no infiltrate or sign of pneumonia as aspiration is a concern. There is some cardiomegaly, and he dose have a diagnosis in his chronic disease list of cardiomyopathy along with a number of other chronic CV conditions.
 Currently he is asymptomatic with this. For some reason, although a cultural sensitivity of the urine was ordered previously, the lab did not carry it out. We discovered today that the lab did not do a culture and sensitivity of the urine. She did start the level one and the sodium tablets as discussed previously. She states his condition is relatively the same

Medical History: P afib, CAD, DM2, hyperlipidemia, hypertension, hypothyroidism, prostate cancer, constipation, squamous cell carcinoma, the mouth/tongue with mets, congestive heart failure, dilated cardiomyopathy

Objective:

Assessment

I42.0 DILATED CARDIOMYOPATHY
 I51.7 Cardiomegaly
 N39.0 URINARY TRACT INFECTION, SITE NOT SPECIFIED

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Visit Date: 06/22/2026

Review of Systems

Positive and pertinent negative responses below. All other areas negative.

Constitutional - Generalized weakness, remains at baseline.

Cardiovascular - Chest x-ray showed negative, which his previous diagnosis. He has no cardiovascular symptoms to report..

Respiratory - She states his cough is a little better. No signs of distress or shortness of breath..

Gastrointestinal - Constipation is managed with current treatment.

Genitourinary - Remains without urinary symptoms.

Skin - G2 side slightly improved.

Psychiatric - Still having episodes of disorientation, she says "maybe slightly better" .

Problem List:

Code	Description	Onset	Status	Status Date
E03.9	HYPOTHYROIDISM, UNSPECIFIED		05/13/2026	Current
	05/13/2026			
E11.65	TYPE 2 DIABETES MELLITUS WITH HYPERGLYCEMIA		05/13/2026	
	Current	05/13/2026		
E43	UNSPECIFIED SEVERE PROTEIN-CALORIE MALNUTRITION			
	06/22/2026	Current	06/22/2026	
K59.00	CONSTIPATION, UNSPECIFIED	06/16/2026	Current	06/16/2026
M62.561	MUSCLE WASTING AND ATROPHY, NEC, RIGHT LOWER LEG			
	05/13/2026	Current	05/13/2026	
M62.562	MUSCLE WASTING AND ATROPHY, NEC, LEFT LOWER LEG			
	05/13/2026	Current	05/13/2026	
N39.0	URINARY TRACT INFECTION, SITE NOT SPECIFIED			06/16/2026
	Current	06/16/2026		
R60.0	LOCALIZED EDEMA	06/22/2026	Current	06/22/2026

Plan

BMI Follow-up Plan: reviewed, discussed

Nutrition Counseling: discussed, education given

Physical Activity Counseling: discussed, education given

cardiomegaly: continue to monitor her symptoms. We did send a cardiology referral previously or mobile cardiology to follow him.

UTI: continue the levaquin for now. We did call the lab and they will be visiting to collect the urine for culture. They called and apologized for eliminating that from the previous order. I did discuss with the daughter a more proper way to collect the urine so we get a more accurate resolved. She's to continue monitoring symptoms, continue with the increase in his fluid intake and notify us for any fever or worsening symptoms.

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Visit Date: 06/22/2026

Caregiver agrees with this and verbalizes understanding

Electronically Signed: Melissa Cole, APRN DNP on/at 06/26/2026 11:51:15

Signed By: Melissa Cole, APRN DNP 06/26/2026 11:51 AM



Alva Butler
5316 FISHERSOUND LN
APOLLO BEACH, FL 33572
No home phone on file

Melissa Cole
4912 West Trapnell Rd Plant City, FL 33566-0128
Phone: (813) 417-4767 Fax: (888) 814-0945
Visit Date: 07/10/2026

DOB: 10/04/1940

85 year old, male

Race: No race saved for patient

Ethnicity: No ethnicity for patient saved

Preferred Language: English

Current Allergies:

NKDA

Current Medications:

Novolog FlexPen U-100 Insulin,
100 unit/mL (3 mL)
acetaminophen, 325 mg
oxycodone, 5 mg
alprazolam, 0.25 mg
thiamine mononitrate (vit B1),
100 mg
tamsulosin, 0.4 mg
rosuvastatin, 40 mg
polyethylene glycol 3350, 17
gram/dose
Nutren 2.0, 0.08 gram-2 kcal/mL
levothyroxine, 50 mcg
isosorbide mononitrate, 20 mg
Flonase Allergy Relief, 50
mcg/actuation
clopidogrel, 75 mg
Eliquis, 5 mg
diltiazem HCl, 90 mg
dapagliflozin propanediol, 5 mg
famotidine, 20 mg
carvedilol, 3.125 mg
megestrol, 400 mg/10 mL (40
mg/mL)
lactulose, 10 gram/15 mL
Senexon-S, 8.6-50 mg
Bactrim DS, 800-160 mg
mupirocin, 2 %
sodium chloride, 1,000 mg
Thick-It
escitalopram oxalate, 5 mg

Lab Results:

Test	Result	Abn Flg	Performed
results	---		07/09/2026
Order	---		07/09/2026
Order	---		07/09/2026
Order	---		07/09/2026
Referral	---		07/09/2026
blood	---		07/09/2026
Urine	---		07/09/2026
Sound	---		07/09/2026

Subjective

Chief Complaint: losing weight, decline in status, weaker

History of Present Illness:

Alva Butler is an 85-year-old male with known diagnoses: P afib, CAD, DM2, hyperlipidemia, hypertension, hypothyroidism, prostate cancer, constipation, squamous cell carcinoma, the mouth/tongue with mets, congestive heart failure, dilated cardiomyopathy. He is being evaluated via telehealth, accompanied by his daughter (primary caregiver) regarding recent decline in status, weakness and weight loss. not strong enough to participate in therapy. They are very concerned and she states he is tired of going to the hospital and refusing to seek acute care for this issue.

1) feels he is not getting enough calories orally and through tube feeding- would like to consider continuous feedings through the night. residuals are low, no nausea / vomiting. Currently on Nutren 2.0 and gets 2 feedings through the day in addition to food intake which is minimal. gets TF supplies from Option care.

2) blood glucose 200-300- covering with sliding scale. feels it is the tube feedings as they have high carbohydrates. this just started recently

3) has new open wounds on sacrum that developed this week. Has HH in there and used to see Woundology. no dressings at this time.

4) believes he is depressed also and would like medication for this, hoping this has something to do with his appetite

No acute distress, they do not need refills.

The patient has severe physical deconditioning related to multiple chronic comorbidities and continues to experience a decline in functional status. He is unable to independently perform essential transfers, including transfers from bed to wheelchair and wheelchair to commode. Due to significant difficulty with weight bearing, severe muscle weakness, declining lower-

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Visit Date: 07/10/2026

extremity strength, and impaired balance, a caregiver cannot safely perform manual transfers. A patient lift is medically necessary to facilitate essential daily transfers within the home, including bed-to-wheelchair and wheelchair-to-commode transfers. Alternative transfer methods, including the use of a transfer board, gait belt, or stand-pivot transfer with caregiver assistance, are not feasible given the patient's current functional limitations. Manual lifting places the patient at significant risk for falls and injury and also poses a substantial risk of musculoskeletal injury to the caregiver, particularly because the patient requires total assistance with transfers.

Medical History: afib, CAD, DM2, hyperlipidemia, hypertension, hypothyroidism, prostate cancer, constipation, squamous cell carcinoma, the mouth/tongue with mets, congestive heart failure, dilated cardiomyopathy

Objective:

Assessment

E11.65 TYPE 2 DIABETES MELLITUS WITH HYPERGLYCEMIA
E43 UNSPECIFIED SEVERE PROTEIN-CALORIE MALNUTRITION
R63.4 ABNORMAL WEIGHT LOSS
R63.0 ANOREXIA
L89.152 PRESSURE ULCER OF SACRAL REGION, STAGE 2
Z43.1 Encounter for attention to gastrostomy
F32.A DEPRESSION, UNSPECIFIED

Review of Systems

Positive and pertinent negative responses below. All other areas negative.
Constitutional - weakness, malaise increasing from baseline, afebrile, apparent weight loss.
Cardiovascular - no chest pain, no swelling, no complaints of palpitations.
Respiratory - Denies shortness of breath, denies cough.
Gastrointestinal - Denies symptoms, poor appetite.
Genitourinary - Denies symptoms.
Musculoskeletal - Weakness as above.
Skin - New stage two ulcer on sacral area.
Psychiatric - Complains of depression.

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 No home phone on file

Visit Date: 07/10/2026

Problem List:

Code	Description	Onset	Status	Status Date
E03.9	HYPOTHYROIDISM, UNSPECIFIED	05/13/2026	Current	05/13/2026
E11.65	TYPE 2 DIABETES MELLITUS WITH HYPERGLYCEMIA	05/13/2026	Current	05/13/2026
E43	UNSPECIFIED SEVERE PROTEIN-CALORIE MALNUTRITION	06/22/2026	Current	06/22/2026
I25.10	ATHSCL HEART DISEASE OF NATIVE CORONARY ARTERY W/O	07/02/2026	Current	07/02/2026
I42.0	DILATED CARDIOMYOPATHY	07/02/2026	Current	07/02/2026
I48.0	PAROXYSMAL ATRIAL FIBRILLATION	07/02/2026	Current	07/02/2026
K59.00	CONSTIPATION, UNSPECIFIED	06/16/2026	Current	06/16/2026
M62.561	MUSCLE WASTING AND ATROPHY, NEC, RIGHT LOWER LEG	05/13/2026	Current	05/13/2026
M62.562	MUSCLE WASTING AND ATROPHY, NEC, LEFT LOWER LEG	05/13/2026	Current	05/13/2026
N18.31	CHRONIC KIDNEY DISEASE, STAGE 3A	07/02/2026	Current	07/02/2026
N39.0	URINARY TRACT INFECTION, SITE NOT SPECIFIED	06/16/2026	Current	06/16/2026
R60.0	LOCALIZED EDEMA	06/22/2026	Current	06/22/2026

Plan

BMI Follow-up Plan: reviewed, discussed

Nutrition Counseling: discussed, education given

Physical Activity Counseling: discussed, education given

decline in status, debility- continued therapy as ordered if he tolerates. Discussed with both caregiver and her brother the possibility of leaning toward palliative care or hospice at this point since he's refusing to seek acute medical care that would be necessary based on his decline in physical status. They are discussing this and will let us know if they want referral. State they will talk to their dad.

Dysphasia/ gastrostomy- the daughter will send me contact information and fax number to the tub feeding company. We will stick with the Neutren for now because of the higher calories. And start continuous feedings through the night. Will continue to bolus in addition to oral food and tape during the day. Discussed some ideas for calorie dense foods throughout the day. Will consider changing to Glucerna if he tolerates this well and his blood sugars remain high.

Diabetes-will continue current treatment, covering with sliding scale throughout the day as often when he wakes up in the morning, his blood sugars are normal.

Stage two pressure ulcer- will send orders for skilled nursing to evaluate and will send referral to wound specialist. Previously, they were seeing Woundology so we will start their first. In the meantime, keep the area clean and dry and continue to reposition him so he is not sitting on that area

Depression.- well go ahead and send a escitalopram to the pharmacy. take medications as prescribed. will titrate up if needed.

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Visit Date: 07/10/2026

Caregivers both agree with this plan and verbalized understanding
Will send into be order when I receive the dme contact information
Will schedule a telehealth visit follow up next week to check on blood sugars and
make sure he gets the new tube feeding supplies that are ordered
Will also touch base about palliative care after they have discussed that and send
referral if they request

Electronically Signed: Melissa Cole, APRN DNP on/at 07/14/2026 11:37:46
Electronically Signed: Melissa Cole, APRN DNP on/at 07/14/2026 12:45:58

Alva Butler DOB: 10/04/1940 85 year old, male
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No home phone on file

Visit Date: 07/10/2026

ADDENDUM

Addendum Subnote

Addendum Created: 7/14/2026 12:45 PM

The patient has severe physical deconditioning related to multiple chronic comorbidities and continues to experience a decline in functional status. He is unable to independently perform essential transfers, including transfers from bed to wheelchair and wheelchair to commode. Due to significant difficulty with weight bearing, severe muscle weakness, declining lower-extremity strength, and impaired balance, a caregiver cannot safely perform manual transfers. A patient lift is medically necessary to facilitate essential daily transfers within the home, including bed-to-wheelchair and wheelchair-to-commode transfers. Alternative transfer methods, including the use of a transfer board, gait belt, or stand-pivot transfer with caregiver assistance, are not feasible given the patient's current functional limitations. Manual lifting places the patient at significant risk for falls and injury and also poses a substantial risk of musculoskeletal injury to the caregiver, particularly because the patient requires total assistance with transfers.

Electronically Signed: Melissa Cole, APRN DNP on/at 07/14/2026 11:37:46

Electronically Signed: Melissa Cole, APRN DNP on/at 07/14/2026 12:45:58

Signed By: Melissa Cole, APRN DNP 07/14/2026 11:37 AM, Melissa Cole, APRN DNP 07/14/2026 12:45 PM



Alva Butler
 5316 FISHERSOUND LN
 APOLLO BEACH, FL 33572
 No home phone on file

Celia Ansah
 4912 West Trapnell Rd Plant City, FL 33566-0128
 Phone: (813) 417-4767 Fax: (888) 814-0945
 Visit Date: 07/02/2026

DOB: 10/04/1940
 85 year old, male
Race: No race saved for patient
Ethnicity: No ethnicity for patient saved
Preferred Language: English
Current Allergies:
 NKDA

Current Medications:
 Novolog FlexPen U-100 Insulin,
 100 unit/mL (3 mL)
 acetaminophen, 325 mg
 oxycodone, 5 mg
 alprazolam, 0.25 mg
 thiamine mononitrate (vit B1),
 100 mg
 tamsulosin, 0.4 mg
 rosuvastatin, 40 mg
 polyethylene glycol 3350, 17
 gram/dose
 Nutren 2.0, 0.08 gram-2 kcal/mL
 levothyroxine, 50 mcg
 isosorbide mononitrate, 20 mg
 Flonase Allergy Relief, 50
 mcg/actuation
 clopidogrel, 75 mg
 Eliquis, 5 mg
 diltiazem HCl, 90 mg
 dapagliflozin propanediol, 5 mg
 famotidine, 20 mg
 carvedilol, 3.125 mg
 megestrol, 400 mg/10 mL (40
 mg/mL)
 lactulose, 10 gram/15 mL
 Senexon-S, 8.6-50 mg
 Bactrim DS, 800-160 mg
 mupirocin, 2 %
 sodium chloride, 1,000 mg
 levofloxacin, 250 mg
 Thick-It

AUDIO/VISUAL TECHNOLOGY UTILIZED.

Lab Results:

Test results	Result	Abn Flg	Performed
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Subjective

Chief Complaint:

History of Present Illness:

07/02/2026

Alva Butler is an 85-year-old male evaluated via telehealth for new-onset left lower extremity numbness and weakness, accompanied by worsening shortness of breath. History was primarily provided by his daughter/caregiver. She reports that over the past day, the patient developed numbness in the left leg to the extent that physical therapy was unable to assist him out of bed due to weakness and inability to sit at the bedside. The patient has a known history of diabetic neuropathy; however, the caregiver states these symptoms are new and significantly different from his baseline.

She denies associated leg swelling, erythema, warmth, coolness, or pain. The caregiver reports that home caregivers had been massaging the affected leg for symptom relief. The patient has no known history of deep vein thrombosis but does have peripheral arterial disease with prior vascular intervention for left lower extremity arterial blockage following a diabetic foot infection. Family reports he was previously informed that additional vascular intervention might eventually be required.

Additionally, the patient has developed worsening shortness of breath with minimal exertion, including during transfers. A recent chest X-ray was reportedly performed, but dyspnea has persisted and is new compared to baseline. Given the combination of unilateral lower extremity symptoms and new dyspnea, there is concern for possible venous thromboembolism.

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Alva is an 85yrs/M with multiple chronic disorders who residing at home with daughter.

History was obtained primarily from the patient's daughter, Deborah, during today's telehealth visit.. today we are reviewing his chest x-ray results.

There's no infiltrate or sign of pneumonia as aspiration is a concern. There is

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some cardiomegaly, and he dose have a diagnosis in his chronic disease list of cardiomyopathy along with a number of other chronic CV conditions. Currently he is asymptomatic with this. For some reason, although a cultural sensitivity of the urine was ordered previously, the lab did not carry it out. We discovered today that the lab did not do a culture and sensitivity of the urine. She did start the level one and the sodium tablets as discussed previously. She states his condition is relatively the same

Medical History: P afib, CAD, DM2, hyperlipidemia, hypertension, hypothyroidism, prostate cancer, constipation, squamous cell carcinoma, the mouth/tongue with mets, congestive heart failure, dilated cardiomyopathy

Objective:

Assessment

R20.0 Anesthesia of skin
 R06.02 Shortness of breath
 R29.898 Other symptoms and signs involving the musculoskeletal system
 E11.40 TYPE 2 DIABETES MELLITUS WITH DIABETIC NEUROPATHY, UNSP
 I73.9 Peripheral vascular disease, unspecified
 Z86.79 Personal history of other diseases of the circulatory system

Review of Systems

Positive and pertinent negative responses below. All other areas negative.

Problem List:

Code	Description	Onset	Status	Status Date
E03.9	HYPOTHYROIDISM, UNSPECIFIED	05/13/2026	Current	
E11.65	TYPE 2 DIABETES MELLITUS WITH HYPERGLYCEMIA	05/13/2026	Current	05/13/2026
E43	UNSPECIFIED SEVERE PROTEIN-CALORIE MALNUTRITION	06/22/2026	Current	06/22/2026

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I25.10	ATHSCL HEART DISEASE OF NATIVE CORONARY ARTERY W/O	07/02/2026	Current	07/02/2026
I42.0	DILATED CARDIOMYOPATHY	07/02/2026	Current	07/02/2026
I48.0	PAROXYSMAL ATRIAL FIBRILLATION	07/02/2026	Current	07/02/2026
K59.00	CONSTIPATION, UNSPECIFIED	06/16/2026	Current	06/16/2026
M62.561	MUSCLE WASTING AND ATROPHY, NEC, RIGHT LOWER LEG	05/13/2026	Current	05/13/2026
M62.562	MUSCLE WASTING AND ATROPHY, NEC, LEFT LOWER LEG	05/13/2026	Current	05/13/2026
N18.31	CHRONIC KIDNEY DISEASE, STAGE 3A	07/02/2026	Current	07/02/2026
N39.0	URINARY TRACT INFECTION, SITE NOT SPECIFIED	06/16/2026	Current	06/16/2026
R60.0	LOCALIZED EDEMA	06/22/2026	Current	06/22/2026

Plan

BMI Follow-up Plan: reviewed, discussed

Nutrition Counseling: discussed, education given

Physical Activity Counseling: discussed, education given

Order STAT venous Doppler ultrasound of the left lower extremity to evaluate for deep vein thrombosis.

Instruct caregivers to stop massaging the affected leg immediately until DVT has been excluded due to the potential risk of clot embolization.

Advise close monitoring of symptoms.

Strongly recommend immediate evaluation in the emergency department if shortness of breath worsens, if chest pain develops, or if there is any further decline in mobility or neurologic status, as urgent evaluation for pulmonary embolism or other cardiopulmonary pathology may be necessary.

Follow up promptly upon receipt of Doppler ultrasound results to determine further management.

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Electronically Signed: Celia Ansah, APRN on/at 07/02/2026 12:01:30

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